

# Journal Editorial Implementation – Batch 1

\*\*Status:\*\* Controlled body-content implementation complete; not published.

## ## 1. What Happens During a Hospice Evaluation? A Family Guide

1. \*\*Previous title:\*\* What Happens During a Hospice Evaluation?
2. \*\*Revised title:\*\* What Happens During a Hospice Evaluation? A Family Guide
3. \*\*Revised body:\*\* Reframed around the family-facing evaluation sequence: first conversation, participants, records and clinical context, questions,
4. \*\*Primary search intent:\*\* Informational – understand a hospice evaluation before deciding.
5. \*\*Core page supported:\*\* `/hospice-care` and `/family-guide`; `/refer` supports professional context.
6. \*\*Contextual internal links added:\*\* `/hospice-care`, `/family-guide`, `/refer`, `/resources/medicare-hospice-benefit`.
7. \*\*Material removed or consolidated:\*\* Removed checklist-like certainty and geographic filler; consolidated family-request/evaluation questions into
8. \*\*Healthcare claims reviewed:\*\* Eligibility is individualized and physician-certified; no prognosis, treatment, dosage, coverage guarantee, or enrol
9. \*\*Final human verification:\*\* Confirm current eligibility wording, evaluation participants, and local referral workflow.
10. \*\*Word count before and after:\*\* 905 → 892 words (JSON lede, section paragraphs, CTA heading, and CTA copy; metadata excluded).

## ## 2. 10 Changes That Can Prompt a Hospice Conversation

1. \*\*Previous title:\*\* 10 Signs It May Be Time to Consider Hospice
2. \*\*Revised title:\*\* 10 Changes That Can Prompt a Hospice Conversation
3. \*\*Revised body:\*\* Retitled and clarified the ten observations as conversation prompts rather than an eligibility checklist; reduced deterministic l
4. \*\*Primary search intent:\*\* Informational – recognize changes worth discussing with a clinician.
5. \*\*Core page supported:\*\* `/hospice-care`.
6. \*\*Contextual internal links added:\*\* `/hospice-care`, Article #1, `/family-guide`.
7. \*\*Material removed or consolidated:\*\* Removed conclusion-like “time to consider” framing and kept disease-specific content out of the general articl
8. \*\*Healthcare claims reviewed:\*\* No diagnosis or eligibility determination; medication and symptom questions remain clinician-directed.
9. \*\*Final human verification:\*\* Confirm the ten observations remain clinically appropriate for the intended audience.
10. \*\*Word count before and after:\*\* 857 → 850 words (same body-count method).

## ## 3. Preparing for Hospice at Home in Thousand Oaks: Questions for Families

1. \*\*Previous title:\*\* Hospice at Home in Thousand Oaks: What Families Can Expect
2. \*\*Revised title:\*\* Preparing for Hospice at Home in Thousand Oaks: Questions for Families
3. \*\*Revised body:\*\* Refocused on household preparation, first visits, caregiver availability, equipment, communication, privacy, supplies, and after-f
4. \*\*Primary search intent:\*\* Informational/local support – prepare for home-hospice logistics.
5. \*\*Core page supported:\*\* `/hospice-thousand-oaks-ca`.
6. \*\*Contextual internal links added:\*\* One strong link to `/hospice-thousand-oaks-ca`, plus `/hospice-care`.
7. \*\*Material removed or consolidated:\*\* Reduced local scenery and commercial selection language; removed broad admission-speed framing.
8. \*\*Healthcare claims reviewed:\*\* No response, admission, service-area, or coverage guarantee; equipment and services remain plan-dependent.
9. \*\*Final human verification:\*\* Confirm local service-area and setting language.
10. \*\*Word count before and after:\*\* 848 → 844 words (same body-count method).

## ## 4. Planning Hospice After Hospital Discharge: A Family Checklist

1. \*\*Previous title:\*\* Can Hospice Begin After a Hospital Discharge?
2. \*\*Revised title:\*\* Planning Hospice After Hospital Discharge: A Family Checklist
3. \*\*Revised body:\*\* Made the transition/handoff the sole job: records, destination, equipment, medications, transportation, named responsibilities, ti
4. \*\*Primary search intent:\*\* Informational/transition – coordinate hospice after discharge.
5. \*\*Core page supported:\*\* `/hospice-care` and `/refer`.
6. \*\*Contextual internal links added:\*\* `/refer`, Article #1, Article #11, `/resources/first-48-hours`.
7. \*\*Material removed or consolidated:\*\* Removed evaluation and post-enrollment duplication; retained only transition-relevant clinical and practical
8. \*\*Healthcare claims reviewed:\*\* No placement or timing guarantee; medication changes remain clinician-directed; emergency language remains cautious.
9. \*\*Final human verification:\*\* Confirm current discharge and referral workflow.
10. \*\*Word count before and after:\*\* 830 → 822 words (same body-count method).

## ## 5. Medicare Hospice Coverage: Questions to Confirm Before Choosing Care

1. \*\*Previous title:\*\* Does Medicare Cover Hospice Care?
2. \*\*Revised title:\*\* Medicare Hospice Coverage: Questions to Confirm Before Choosing Care
3. \*\*Revised body:\*\* Reframed as a plain-language orientation and question checklist, with early routing to the authoritative Medicare resource and Mec
4. \*\*Primary search intent:\*\* Informational – understand benefit basics and identify questions requiring confirmation.
5. \*\*Core page supported:\*\* `/resources/medicare-hospice-benefit`.
6. \*\*Contextual internal links added:\*\* `/resources/medicare-hospice-benefit` early; official Medicare.gov hospice guidance; `/family-guide`, `/hospice
7. \*\*Material removed or consolidated:\*\* Reduced duplicate comprehensive-benefit framing and replaced broad statements with cautious, patient-specific
8. \*\*Healthcare claims reviewed:\*\* Medicare Part A, certification, benefit-related services, cost sharing, respite, and plan-of-care wording reviewed
9. \*\*Final human verification:\*\* Confirm current Medicare.gov wording, cost-sharing amounts, benefit periods, and related/unrelated service rules befor
10. \*\*Word count before and after:\*\* 853 → 847 words (same body-count method).

## ## Batch-level cannibalization check

The five articles retain separate jobs:

- \*\*Article #1\*\* owns the family evaluation process; Article #12 remains a separate source file but is not expanded or modified.
- \*\*Article #2\*\* owns observable changes and conversation prompts, while the legacy myths article remains misconception correction and condition guides
- \*\*Article #3\*\* supports the Thousand Oaks city page with practical home preparation and does not become a second local landing page.
- \*\*Article #4\*\* owns the hospital-to-hospice handoff and stops before the evaluation or first 48 hours.
- \*\*Article #5\*\* supports the Medicare resource and does not attempt to replace its comprehensive coverage detail.

Only the five authorized JSON records and their five matching preview pages were revised, plus this report. The derived site search index was regenerat